
BRIGHTKIDCO EMAIL ECOSYSTEM
AVATAR: AUTISTIC PARENT
DOC 4 OF 6 — LEVEL 3 PROFILE
(Non-verbal/minimal verbal, high support needs, 6-10+ years)
VERSION 2.0 — END-LEVEL FOR MARKETER WORKFLOW

This doc is a sub-profile within the Autistic Parent avatar.
Read DOC 1 (Master Foundation) first.

Audience share: ~25% of the Autistic Parent audience.
Selection: Pop-up option "Non-verbal or high support needs"
→ Tag: avatar_level3

IMPORTANT: This is the MOST EMOTIONALLY SENSITIVE audience.
Every mail is written with particular care. This mom often
has years of trauma experience with failed attempts, stigma,
and comparison pain. Language that works for Level 1 or 2
can do damage here.

DIGNITY-FIRST IS NON-NEGOTIABLE.

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PART A: WHO IS THE LEVEL 3 MOM

1. DETAILED AVATAR PORTRAIT

THE LEVEL 3 MOM IN PORTRAIT:

Imagine Maria. She's 42, lives in a suburb in Ohio. Her son Daniel is 9, diagnosed Level 3 ASD, minimal verbal — he uses an AAC tablet for communication, has ~30 functional words, but can't form sentences.

Daniel has never been potty-trained. He wears specialty diapers size XL — she orders them through a Medicaid Waiver because regular pull-ups no longer fit. He's 130 cm tall, 35 kg. Maria packs 3 change diapers in his school bag for special ed each morning.

Maria has tried — really tried:

- 3 years of ABA therapy (40 hours/week until age 5)
- A 3-week residential potty program at a specialty center (Daniel was 5)
- BCBA protocol with scheduled sits since he was 4
- OT with deep pressure and sensory integration
- Picture schedules, Velcro boards, social stories
- Specialty toilet seats with side bars
- Music therapy approach with bathroom songs
- Private potty consultant (\$2500, no change)

Nothing has led to independent initiation. Daniel sits on the toilet when brought. He sometimes goes. But he never signals, never initiates, never feels — or if he does, he can't show it.

Maria decided 2 years ago "to stop actively training and focus on dignity." She told the pediatrician. He agreed. Daniel's special ed school supports with diaper changes as part of his IEP.

But: Daniel turns 10 in 6 months. He's outgrowing the largest specialty diaper size. The next option is adult diapers. Maria has already seen the box in the store. She

cried.

She has no Pre-K worry. She has no "tried everything" hope anymore. She has accepted: maybe he'll never train. If he does one day, it will be a miracle.

But she still searches. Not actively, more passively. A Reddit post about autism + 9-year-old + body-signal training. A comment from another mom in a special needs Facebook group. A single post in a newsletter that mentions "Maureen Bennie's son, 9.5 years, finally trained."

She clicks out of pure exhaustion. Little expectation. But she clicks.

THAT IS THE LEVEL 3 MOM.

CORE CHARACTERISTICS:

- HOPE is accepted as lost (but not entirely dead)
- Resignation is her default mode
- "Maybe never" is permitted reality
- Active searching she doesn't do — she reacts to signals
- When she buys, it's a last attempt
- She is hyper-sensitive to language
- Comparison pain is 6-9 years deep
- She is caregiver, not "potty-training mom"
- She has accepted identity as mom of a severely disabled child
- She reads slowly, carefully
- When mail hurts her, she blocks immediately

2. DEMOGRAPHICS & CONTEXT

AGE & FAMILY:

- Mom: 35-48 years
- Often 2-3 children, autistic child is often not the first
- 20-25% single moms (higher than other levels)
- Other siblings often already teenagers, accept family burden

- Sometimes: grandparents live with or help daily

GEO DISTRIBUTION:

- USA: 70% (Medicaid geography concentrated)
- UK: 15%
- AU/NZ: 10%
- Canada: 5%

INCOME LEVEL:

- Lower-middle income dominant
- Working-class: ~35%
- Lower income / Medicaid: ~40% (significantly higher than Level 1+2)
- SSI disability for child: common
- Career often entirely given up for caregiving

EMPLOYMENT STATUS:

- 30% working (often part-time, often lower-qualified after career change)
- 60% stay-at-home/full-time caregiver
- 10% highly qualified with massive childcare setup
- College-educated: ~55%

MEDICAID/INSURANCE:

- ~50% use Medicaid Waiver for specialty diapers
- ~30% use Medicaid for school therapy
- ~70% have insurance battles for BCBA therapy
- Very financially sensitive

CHILD COMORBIDITIES (typical Level 3):

- Sensory Processing Disorder: ~98% (severe)
- Intellectual Disability (sometimes): ~50%
- Speech delays (often non-verbal): ~80%
- GI issues (chronic): ~60-70%
- Seizure disorders: ~25%
- Anxiety/Self-injury behaviors: ~40%
- Sleep disorders: ~70%
- Feeding issues: ~50%

SCHOOL STATUS:

- Special needs school: ~70%
- Mainstream with 1:1 aide: ~25%
- Homeschool: ~5% (sometimes burnout consequence)
- IEP with diaper support: ~95%

DIAGNOSIS STATUS:

- ~95% formally diagnosed Level 3 (autism severity scale)
- ~5% Level 2 diagnosis but functions like Level 3

THERAPY HISTORY (typical):

- 3-7+ years BCBA + ABA
- 3+ years OT
- 3+ years Speech
- Sometimes: residential programs (1-3 weeks intensive)
- Sometimes: private potty consultants

3. THE LEVEL 3 CHILD IN DETAIL

AGE:

- 6-10+ years dominant
- Sometimes younger 5, sometimes older 12+
- Teenagers exist in this audience too

VERBAL PROFILE:

- Non-verbal or minimal verbal
- Often AAC device (Augmentative and Alternative Communication)
- 0-50 functional words typical
- Can often NOT verbally communicate needs
- Speech when present is often scripted, echolalic

COGNITIVE:

- Highly variable — some intellectual disability, some not
- Often understand more than they can express
- Routine sensitivity extreme
- Generalization very limited
- Learns new skills slowly, but persistent

SENSORY:

- SEVERE sensory differences
- Hyper-sensitive AND/OR hypo-sensitive (often both simultaneously)
- Sensory triggers can cause self-injury
- Bathroom is often major sensory hell

- Toilet flushing, lights, echo, temperature — all potential triggers

BEHAVIOR:

- Meltdowns regularly (daily to multiple times daily)
- Self-stimulatory behaviors prominent
- Sometimes: self-injury when frustrated
- Special interests very deep, very narrow
- Social interaction very limited or specific

POTTY TRAINING PROFILE — LEVEL 3 SPECIFIC:

THE TYPICAL LEVEL 3 POTTY SITUATION:

The child:

- HAS NEVER been potty-trained, BUT actively tried
- SITS on the toilet (with years of acclimation)
- INITIATES never, signals never
- HOLDS bowel movements often (withholding extreme)
- HAS massive sensory triggers in the bathroom
- CANNOT show when he needs to
- ACCIDENT PATTERN: constant, unpredictable
- EVENTUALLY adult diaper sizes

THE LEVEL 3 REALITY:

"He's 9. He's been in pull-ups his entire life. Specialty diapers now. The next size up is adult diapers. I'm preparing myself."

That is not defeat. That is realism.

TYPICAL LEVEL 3 ACCIDENTS:

- Complete diaper soils every 1-2 hours
- Multiple laundry loads per day
- Skin issues from chronic moisture
- Public outings extremely limited
- Vacations practically impossible
- Smell becomes family burden

4. WHAT DISTINGUISHES LEVEL 3 FROM OTHER LEVELS

VS LEVEL 1:

- Level 3 has 5+ years of training attempts (Level 1: 1-3 attempts)
- Level 3 child non-verbal (Level 1: verbal)
- Level 3 no Pre-K pressure (Level 1: critical)
- Level 3 has accepted resignation (Level 1: still has hope)

VS LEVEL 2:

- Level 3 is beyond method cycling (Level 2 in the middle)
- Level 3 child older (6-10+ vs. 5-7)
- Level 3 has accepted "maybe never" (Level 2: not yet)
- Level 3 Medicaid reality heavier (40% vs 25%)
- Level 3 GI issues more common
- Level 3 comorbidities more severe
- Level 3 school setting: often special needs (Level 2: mainstream with support)

CRITICAL IMPLICATIONS FOR MARKETING:

Level 3 mails must NEVER:

- Use "hopeful" language as trick
- Draw comparisons with Level 1/2 successes
- Use "easy" or "quick"
- Use Pre-K or school pressure as lever
- "Your child can do this!" cheerleading
- Standard mom voice (she is different)

Level 3 mails MUST:

- Dignity in every sentence
- "Mini-wins count" framing
- Acknowledge late-trainer reality
- Understand specialty sizes
- Reference adult diapers (carefully)
- Medical team respect

5. THE RESIGNATION REALITY

WHAT RESIGNATION MEANS FOR LEVEL 3:

It is NOT giving up.

It is NOT "I don't care anymore."

It is NOT "He'll never train."

It IS: acceptance after years of pain.

It IS: realistic peace with what is.

It IS: energy preservation for what's actually possible.

LEVEL 3 MOM'S WORDS ON RESIGNATION:

"I stopped trying actively 2 years ago. We focus on dignity now."

"He'll get there when he gets there. Or he won't. Either way, he's my son."

"I don't 'hope' anymore. I just keep him clean and safe."

"Maybe one day. Maybe never. We're at peace either way."

"I'm not in a rush. He's not in a rush. We're managing."

HOW MAILS MUST ACKNOWLEDGE THIS:

DO:

✓ "If you've moved past active training — that's not failure"

✓ "Many families redirect energy to dignity-focused management"

✓ "If something shifts, it shifts. If not, you've been doing the right thing"

✓ "This isn't about reigniting the fight"

DON'T:

✗ "Don't give up!"

✗ "Your child can still do this!"

✗ "It's never too late!" as SOLE message

✗ "Re-energize your training!"

✗ Cheerleader language

THE BALANCE:

Mails should gently suggest: "Here is something new, in case you're ever curious — no pressure."

PART B: PSYCHOLOGY OF THE LEVEL 3 MOM

6. THE 4 PHASES FOR LEVEL 3

For Level 3, PHASE 4 is dominant. Phase 3 is second most common. Phases 1-2 are very rare in this audience.

PHASE 1 FOR LEVEL 3: VERY RARE

Phase 1 (Slow Realization) is almost non-existent for Level 3. When a child is diagnosed later as Level 3, the mom is often already deep in therapy world. She rarely comes to BrightKidCo in Phase 1.

CONVERSION PROBABILITY: <1%

PHASE 2 FOR LEVEL 3: RARE

Child 4-5 years

WHAT HAPPENS:

- Diagnosis is there, severity slowly becomes clear
- First BCBA recommendations for toileting
- Standard methods not even attempted (obviously not fitting)
- Begins specialty approaches

EMOTIONAL STATE:

- Deep shock at diagnosis severity
- Grief dominant

- Focus on speech, sensory, sleep — not potty

EMAIL IMPLICATION:

- Very gentle, almost no sales energy
- Education mode
- Brand awareness planting

CONVERSION PROBABILITY: ~3-5%

PHASE 3 FOR LEVEL 3: METHOD CYCLING (SECONDARY DOMINANT)

Child 5-7 years

WHAT HAPPENS:

- Has tried all specialty methods
- ABA, OT, residential programs, private consultants
- Some small wins, but never "fully trained"
- GI issues have developed
- School setting: special ed or mainstream with 1:1
- Family has integrated diaper routine as normality
- But: still actively trying — has not resigned

EMOTIONAL STATE:

- Skepticism very high
- "I've tried everything" has 5+ years of substance
- Science is respected when it's VERY specific
- Begins searching for interoception (often via OT)
- Exhaustion dominant but not yet accepted

EMAIL IMPLICATION:

- Science + Dignity-First
- Late-trainer stories begin to become relevant
- "Add alongside what you do" language
- Very long timelines (12+ weeks)

CONVERSION PROBABILITY: ~5-10%

PHASE 4 FOR LEVEL 3: RESIGNATION FORK (PRIMARY DOMINANT)

Child 7+ years

THIS IS THE MAIN FOCUS FOR LEVEL 3 MARKETING.

Most Level 3 subscribers are in Phase 4.

WHAT HAPPENS:

- Has distanced herself from active training (often 1-3 years)
- Specialty diapers routine for years
- School setting accepts diaper support
- Lives with "maybe never" reality
- BUT: a trigger event has reached her
- Trigger could be:
 - Adult diaper size transition approaching
 - School change (Elementary to Middle School)
 - Puberty approaching
 - Other family member experienced trauma from inability
 - Another mom shared "late trainer" success
 - Newsletter or Reddit post about interoception
 - Insurance/Medicaid change
 - Sibling proposed family trip that's difficult

EMOTIONAL STATE:

- Deep exhaustion
- Little active hope
- But: latent curiosity
- Hyper-vigilant for language (mails with fake optimism → instant block)
- Reads slowly and thoroughly
- Decides slowly (often weeks)

DECISION TRIGGERS:

- Late-trainer story (Maureen Bennie 9.5 years)
- Dignity-respectful mechanism explanation
- "Add alongside specialty diapers" framing
- Very long timeline promises
- 60-day guarantee as no-pressure experiment

EMAIL IMPLICATION:

- Mails are gentle, slow, respectful
- Late-trainer stories prominent
- Mini-wins defined as REAL success (not "fully trained")
- Acknowledge specialty sizes
- Medicaid sensitivity high
- Permission-not-to-buy in every mail
- Accept very long decision period (often 30+ days)

CONVERSION PROBABILITY: ~4-8%

CAUTION: Lowest conversion rate of all levels. But: often very loyal after purchase, highest LTV.

PHASE SUMMARY LEVEL 3:

Phase Share of L3 Audience Conversion Rate		
----- ----- -----		
1	~2%	<1%
2	~10%	3-5%
3	~33%	5-10%
4	~55%	4-8%

→ Phase 4 is the dominant area for Level 3. Email strategy should be precise here.

7. SYMPTOM CLUSTERS (6 CLUSTERS)

These 6 clusters are Level 3 specific. In every mail there should be 2-3 as recognition anchors — but subtly, without emotional overload.

SYMPTOM CLUSTER A: COMPLETE SIGNAL ABSENCE

EXACT FORMULATIONS FROM RS:

"He's never shown a single sign of feeling wet or dirty."

"She has no awareness of bodily functions at all."

"He's been in pull-ups for 9 years and never reacted to a single wet diaper."

"There are no readiness signs. There never have been."

"He doesn't pull at his diaper, doesn't ask for changes,

doesn't react to soiled."

"Neither boy was bothered or even seemed to notice if they are wet or dirty."

WHAT IT MEANS:

The interoceptive system either sends no signal, or the signal cannot be translated to awareness. This is not behavioral — it's neurologically fundamental.

EMAIL IMPLICATION:

Mom has often spent years searching for "readiness signs" that never came. This cluster is deeply painful.

SYMPTOM CLUSTER B: SENSORY OVERLOAD BATHROOM

EXACT FORMULATIONS FROM RS:

"He won't even enter the bathroom. The lights, the sound, everything is too much."

"She has meltdowns just from the toilet seat texture."

"He needs noise-cancelling headphones in the bathroom."

"The flushing sound triggers self-injury."

"She has a fear of falling into the toilet — won't sit."

"He won't go in public bathrooms — we're trapped at home."

WHAT IT MEANS:

Bathroom = sensory war zone. Training cannot start before sensory acclimation happens.

EMAIL IMPLICATION:

Sensory awareness in mail language is critical. Acknowledge bathroom specifics.

SYMPTOM CLUSTER C: WITHHOLDING + SEVERE GI ISSUES

EXACT FORMULATIONS FROM RS:

"He withholds bowel movements for 5-7 days at a time."

"She's been hospitalized for impaction twice."

"He's on a daily Miralax dose for years."

"GI specialist appointments every 3 months."

"He has overflow incontinence — soils without realizing."

"She's been on a strict diet for 4 years to manage GI."

WHAT IT MEANS:

GI complications are not periphery — they are often the central medical theme. Withholding is sensory + fear cycle.

EMAIL IMPLICATION:

Careful: NO promises for GI healing. Medical team as primary, body-signal as adjunctive.

SYMPTOM CLUSTER D: COMMUNICATION LACK

EXACT FORMULATIONS FROM RS:

"He can't tell me when he needs to go — he's non-verbal."

"She uses AAC for food and toys but can't request bathroom."

"He has 30 words. Bathroom isn't one of them."

"She points to things but can't communicate body needs."

"He hits his diaper sometimes — I don't know if that's a signal."

"There's no way for him to let me know."

WHAT IT MEANS:

Even when child feels something, they can't communicate.

This is a double barrier: signal + communication.

EMAIL IMPLICATION:

Important: body-signal training targets the feeling (signal), not the speaking. Mom must understand: this is just step 1.

SYMPTOM CLUSTER E: PHYSICAL RESISTANCE

EXACT FORMULATIONS FROM RS:

"He kicks me in the face when I try to change him."

"My 6 year old son kicked me in the face today, while having a meltdown when I changed his diaper."

"She self-injures when I try to put her on the toilet."

"He hits his head when we're in the bathroom."

"It takes two adults to change him now that he's bigger."

"He's stronger than me — I can't physically guide him anymore."

WHAT IT MEANS:

Physical resistance is real and results from sensory overload + communication lack + frustration. This mom often has daily physical confrontations.

EMAIL IMPLICATION:

Very emotional and painful. Mails should acknowledge this without dramatizing. NO promise that the product solves this resistance — it cannot.

SYMPTOM CLUSTER F: SIZE AND PROGRESSION FEAR

EXACT FORMULATIONS FROM RS:

"He's outgrown size 7. We're moving to specialty diapers."

"The next step is adult diapers. I'm not ready."

"She's getting too big for me to carry to the changing table."

"He's 65 pounds. I can't lift him anymore."

"Where do you even buy adult diapers for kids?"

"I've started looking at adult-size pull-ups online."

WHAT IT MEANS:

Size progression shows permanence of the situation. Adult diapers feel like "final defeat."

EMAIL IMPLICATION:

Very sensitive. Acknowledging adult diapers OK, but NEVER as sales pressure. This is trauma material.

USE CASE FOR THE 6 CLUSTERS:

In Level 3 mails, 2-3 of these clusters should appear as recognition anchors, but never all 6. Too much pain at once = block.

Example combinations:

Welcome E1: Cluster A + Cluster F (gently)

Welcome E2: Cluster A + Cluster B (mechanism)

Cart Aband: Cluster A + Cluster D (validation)

FAQ: Cluster B + Cluster C (sensory + GI)

8. VERBATIM LANGUAGE BANK — LEVEL 3 SPECIFIC

These quotes come directly from Level 3 family sources. They are particularly raw and painful. Never rewrite.

CATEGORY 1: HOW LEVEL 3 MOMS DESCRIBE THEIR CHILD

"He's 9. He's been in pull-ups his entire life."

"She's non-verbal, has Level 3 autism, and has never been toilet-trained."

"My son has GI issues, sensory issues, and is completely non-verbal. Potty training isn't even on the radar."

"He hits his head against the wall when I try to take him to the toilet."

"She wears specialty diapers now. Adult-sized when she outgrows these."

"He's never given me a signal in 8 years."

"My 6 year old son kicked me in the face today, while having a meltdown when I changed his diaper."

"He's outgrown size 7 pull-ups. We're moving to specialty diapers."

"She's 8 and has chronic constipation. Daily Miralax for 4 years."

"He doesn't realize he needs to go. He doesn't realize he's gone."

"She has overflow incontinence — soils without realizing."

CATEGORY 2: HOW LEVEL 3 MOMS DESCRIBE THE JOURNEY

"We did 40 hours of ABA a week for 2 years. He's still in pull-ups."

"We even did a residential program for 3 weeks. He's back in pull-ups within a month of home."

"His BCBA has worked with him for 6 years. No initiation."

"We spent \$2500 on a private potty consultant. Same result."

"I've tried everything. I mean it this time, not as a phrase."

"We stopped active training 2 years ago to focus on dignity."

"I've made peace with maybe never."

"I'm not in a hurry anymore. I'm just trying to keep him healthy."

CATEGORY 3: HOW LEVEL 3 MOMS EXPRESS SELF-DOUBT

"Maybe his autism is too severe."

"I think we missed the window."

"He'll need diapers forever. I'm preparing myself."

"Maybe I should have started sooner. Or differently. Or with a different therapist."

"I don't know what 'success' would even look like at this point."

CATEGORY 4: HOW LEVEL 3 MOMS DESCRIBE PROGRESS (WIN MOMENTS — VERY DIFFERENT)

For Level 3, wins are different. They are smaller, slower, often sensory:

"He let me clean him without biting today."

"She didn't melt down when I changed her."

"He paused. He felt something."

"She stayed on the toilet for 5 minutes without screaming."

"He went 6 hours without a diaper change. That's a record."

"He looked down at the wet spot for the first time in 9 years."

"She tolerated the bathroom for 10 minutes today."

"He let me wipe him without kicking."

IMPORTANT: These wins are not "training success" — they are often sensory tolerance steps. Mails must understand this.

CATEGORY 5: HOW LEVEL 3 MOMS TALK ABOUT REALITY

"This is our life. Pull-ups, changes, accidents, medical appointments."

"School handles changes during the day. I do night."

"We've structured our life around this. Limited outings, specific timing."

"My other kids have learned to live with it. They're patient."

"My partner and I split the changes 50/50. We have a system."

"It's not a battle anymore. It's just life."

CATEGORY 6: WHAT LEVEL 3 MOMS GOOGLE

"non-verbal autism potty training adult"

"severe autism still in pull-ups age 8"

"adult diapers for autistic child age 9"

"medicaid pull-ups autism level 3"

"my 10 year old won't potty train autism"

"specialty diapers for big kids autism"

"interoception severe autism"

"autism level 3 toilet training success stories"

"late toilet training autism 9 years old"

"my non-verbal child has never used the toilet"

9. CONVERSION TRIGGER MAP FOR LEVEL 3

When exactly does the Level 3 mom decide to buy?

IMPORTANT: Conversion triggers for Level 3 are often

EXTERNAL events (reality forces re-engagement), not internal motivation.

PRIMARY TRIGGER #1: ADULT DIAPER SIZE APPROACHING

What she thinks: "He's outgrowing the largest pull-up size. Adult diapers are next. I have to try one more thing."

Mail lever: "It's not too late" + Maureen Bennie 9.5 + late-trainer success stories

Investment readiness: Medium-High
Timeline tolerance: Very long

PRIMARY TRIGGER #2: SCHOOL TRANSITION

What she thinks: "Middle school starts next year. The diaper support won't transfer easily. Other kids will notice."

Mail lever: Privacy + Late-Trainer + Dignity

Investment readiness: High
Timeline tolerance: Long

PRIMARY TRIGGER #3: PUBERTY APPROACHING

What she thinks: "She's almost 10. Period might come. We're not ready for that on top of this."

Mail lever: Hygiene + Late-Trainer stories with older kids

Investment readiness: Medium-High (often grief-driven)
Timeline tolerance: Very long

PRIMARY TRIGGER #4: ANOTHER MOM'S LATE-TRAINER STORY

What she thinks: "I just read about a 9-year-old who finally trained. Maybe..."

Mail lever: Specific Late-Trainer testimonials, Maureen Bennie

Investment readiness: Medium

Timeline tolerance: Long

PRIMARY TRIGGER #5: GI CRISIS REPEATED

What she thinks: "Another hospitalization for impaction. Something has to give."

Mail lever: Mechanism for GI reduction (carefully!), "this isn't a cure but may stabilize"

Investment readiness: High (medical-driven)

Timeline tolerance: short (crisis mode)

SECONDARY TRIGGER: SIBLING SUGGESTION

What she thinks: "My older daughter mentioned this. She read about it."

Mail lever: Reduce social isolation, family relief

WHAT SHE DOES AFTER TRIGGER:

1. Very slow, careful research (weeks!)
2. Reads multiple sources, compares
3. Bookmarks many pages
4. Hesitates long at pop-up
5. Eventually clicks out of "maybe" feeling
6. Welcome flow starts
7. 14-30 days decision period (significantly longer than other levels)
8. Purchase or drop-off

EMAIL IMPLICATION:

- Welcome flow designed very long (decision period 30+ days)
- Late-trainer stories prominent from start
- NO urgency, never
- Science + dignity in balance
- Reply encouragement never pushy

PART C: TESTIMONIAL & PROOF STRATEGY

10. TESTIMONIAL PATTERNS FOR LEVEL 3

CHILD PROFILE IN TESTIMONIAL:

- Age 7-12 years (older than other levels)
- "Diagnosed Level 3" or "high support needs"
- Non-verbal or minimal verbal
- 5+ years of failed attempts
- BCBA / OT / Speech / GI specialist history
- Optional: AAC device mentioned
- Optional: Medicaid context
- Specialty diapers prominent

TIMELINE IN TESTIMONIAL:

- 8-16 weeks to first mini-wins
- 4-12 months to substantial change
- Acknowledged: "still not fully trained but..."
- Realistic: regression cycles normalized
- Puberty / adult diapers often as background

LANGUAGE PATTERNS:

- "After 5+ years of trying everything..."
- "We had stopped actively training..."
- "My son is non-verbal, doesn't sign..."
- "We didn't expect a miracle, just hoped..."
- "Week 8 was the first time he paused..."
- "Still in pull-ups for night, but..."
- "We're 6 months in. He's not 'trained' but..."

DETAILS THAT BUILD TRUST:

- 5+ years history
- Specific therapy hours
- Comorbidities mentioned (GI, sensory, communication)
- Adult diaper size context
- Maureen Bennie-style "late trainer" echo
- Honest "not fully trained" + "but..."
- Mini-win definition redefined

11. SAMPLE TESTIMONIALS — LEVEL 3 (MESSY-REAL)

SHORT TESTIMONIAL (for mail inserts, ~5-7 sentences):

"My son Daniel is 9, Level 3, non-verbal. We had stopped actively potty training 2 years ago — focused on dignity and management.

I tried the 3+3 bundle as a 'why not' last summer. Week 1, he tolerated wearing them during the day. Week 8, he paused after wetting. Week 14, he initiated walking to the bathroom once.

We're still in pull-ups at night. He's not 'trained.' But something his nervous system never registered in 9 years — it's registering now."

— Maria, mom of Daniel, 9yo Level 3 non-verbal

MEDIUM TESTIMONIAL (for Welcome Education, ~12-18 sentences):

"My son is 8, Level 3, minimal verbal — uses an AAC device for ~30 functional words. Bathroom isn't one of them.

We did 5 years of ABA. 3 years of OT. He completed a 3-week residential potty program at a specialty center when he was 6 — came home and was back in pull-ups within a month.

I had stopped actively training around his 7th birthday. We focused on managing GI issues (chronic withholding, daily Miralax), keeping him clean, supporting his school day. School handles changes during the day. I do mornings and nights.

A friend in our autism community mentioned BrightKidCo. I bought the 3+3 expecting nothing. Maybe I was just curious. Maybe I needed to do 'one more thing' before he turned 9 and I had to size up to adult diapers.

Week 1: he tolerated wearing them during play, 1-2 hours at a time. That was new — usually he'd self-injure with unfamiliar clothing.

Week 6: he paused after wetting. Looked down. That was the first time in 8 years I'd seen any acknowledgment.

Week 12: he initiated walking toward the bathroom once. Once. Not when he needed to go — just walked in and stood there. Then left. But it was orientation. New orientation.

We're now 7 months in. He's not 'fully trained.' We still do pull-ups for school and night. But during the day at home, he wears the underwear. He pauses. He sometimes walks to the bathroom. We've reduced his Miralax dose by half — his GI is doing better.

I don't know what comes next. But I know his body is finally registering something it never could before. That's worth something."

— Lisa, mom of [name], 8yo Level 3, minimal verbal

LONG TESTIMONIAL (for PDP, FAQ mails, 20+ sentences):

If marketer needs a long Level 3 testimonial, here is structure:

PARAGRAPH 1: Child profile + diagnosis + years of history

PARAGRAPH 2: What all we tried (detail!) — BCBA, OT, residential, consultants

PARAGRAPH 3: When we stopped actively training + why

PARAGRAPH 4: What brought us to try again (external trigger)

PARAGRAPH 5: Skepticism at purchase + minimal expectations

PARAGRAPH 6: Week-by-week very small wins (sensory tolerance first, signal later)

PARAGRAPH 7: Where they are now + honest "still managing, but..."

PARAGRAPH 8: Recommendation (peer-to-peer, with dignity)

PROOF RANKING FOR LEVEL 3:

#1 STRONGEST PROOF: LATE-TRAINER TESTIMONIAL

"My 9.5 year old finally trained. We had given up."
"Maureen Bennie's son was 9 when something clicked."

Why: Mirrors her reality. "Even at this age, possible."

#2 SECOND STRONGEST: NON-VERBAL CHILD WITH MINI-WIN

"He's non-verbal. He's never told me anything about bathroom. Week 8, he paused. Just paused."

Why: Mom thinks "my non-verbal kid is the same." Specificity builds trust.

#3 THIRD STRONGEST: MEDICAL/GI VALIDATION

"His GI doctor noticed the improvement. We reduced Miralax by half."

Why: Medical authority = trust. GI is often central.

#4 FOURTH STRONGEST: MULTI-YEAR JOURNEY TESTIMONIAL

"After 5 years of ABA and 2 years of giving up, we tried this..."

Why: Validates her timeline. "She's not the only one with years of history."

#5 FIFTH STRONGEST: KELLY MAHLER + INTEROCEPTION RESEARCH

Why: She often knows Mahler, but science is secondary to real-life stories for Level 3.

#6 SIXTH STRONGEST: VOLUME STATS (16,511 Reviews)

Why: Least effective for Level 3 — she has seen "many other" products that didn't help.

PROOF PLACEMENT IN MAILS:

Welcome E1: Maureen Bennie anchor subtly
Welcome E2: Non-verbal kid testimonial
Welcome E3: Late-trainer story prominent
Welcome E4-5: Kelly Mahler + Mechanism
Welcome E6-8: Multi-year journey testimonials
Cart Recovery: Maureen Bennie + 60-day guarantee
FAQ mails: Multiple late-trainer stories
Post-purchase: Mini-win definition + validation

PART D: TONALITY & CALIBRATION

13. TONALITY CALIBRATION PER ELEMENT

HOPE LEVEL: MINIMAL

- NO "this will work" language
- "This may add one piece worth trying" works
- "Late is not never" as anchor, sparingly
- Realistic, not hopeless

REALISM LEVEL: VERY HIGH

- "Some kids take 12 weeks. Some take 6 months. Some never fully train."
- Acknowledge regression cycles as completely normal
- "Not every kid responds. 60 days to know."
- "Mini-wins count more than fully-trained claim"

URGENCY LEVEL: ZERO

- NO Pre-K pressure
- NO school pressure as lever
- NO "running out of time"
- Adult diaper reference only as reality, not sales pressure

SCIENCE DENSITY: MEDIUM-HIGH

- Kelly Mahler yes, but combined with real-life stories
- SPARK stat OK, but secondary
- Interoception very OK (she often knows it)
- BCBA-respectful language throughout
- Specific multi-year therapy acknowledgment

EMPATHY INTENSITY: VERY HIGH

- She has deeply wounded history
- "I see how exhausting these years have been"
- Validate specific situations (sensory meltdowns, withholding, physical resistance)
- Deep observation > shallow sympathy
- NO cheerleading

TESTIMONIAL LENGTH: LONG + MESSY

- Level 3 mom reads very carefully
- 12-18 sentences per testimonial ideal
- Real-life "still not fully trained" language important
- Mention multiple wins
- Honest "we still do pull-ups at night"

CTA AGGRESSIVENESS: VERY LOW

- "Worth considering when you're ready"
- "60-day guarantee, no commitment"
- "If now isn't right, the product will be here"
- NO urgency tricks
- Permission-not-to-buy in EVERY mail

GENERAL ENERGY:

- Slow, deliberate, patient
- Like an experienced special needs mom talking to Maria
- Like a respectful nurse
- Like a late-night Reddit comment from a mom who understands
- Very calm, no sales urgency

14. HOPE VS REALISM BALANCE (MINIMAL HOPE)

THE LEVEL 3 BALANCE:

20% HOPE / 80% REALISM

Hope language (very sparingly):

- "It may add one piece to what you're already doing"
- "Some families see real change. Some don't. Both happen."
- "Worth a try, given everything you've already done"

Realism language (dominant):

- "Some kids take 12+ weeks. Some 6+ months."
- "Not all kids respond. You'll know within 60 days."
- "Wins look different at this stage — pause, look, tolerate"
- "Still in pull-ups at night, even at month 6 — that's normal"
- "We're not promising training. We're offering a missing piece."

WHY 20/80?

Level 3 mom is deeply wounded by hope selling. Any hope language that feels off = instant block.

But: complete hope absence = "why try then?"

Balance is: enough realism to build trust, just enough hope to motivate one more try.

15. DIGNITY-FIRST LANGUAGE

CRITICAL FOR LEVEL 3.

EVERY mail must have dignity in every sentence.

DO LANGUAGE:

- ✓ "Your child"
- ✓ "He is doing the best his nervous system allows"
- ✓ "Whatever progress means for your family is enough"
- ✓ "Your son's identity is intact"

- ✓ "Your daughter's wellbeing matters most"
- ✓ "Dignity in care, dignity in process"
- ✓ "Whether he trains in 8 weeks or never, you've been doing the right thing"
- ✓ "We respect your reality"

DON'T LANGUAGE:

- ✗ "Your child will be normal"
- ✗ "Your kid can do this!"
- ✗ "Cure" / "Fix" / "Solve"
- ✗ "Help your child get better"
- ✗ Comparison to neurotypical kids
- ✗ "Save him from..."
- ✗ "Free her from diapers"
- ✗ "Become like other kids"

KEY MESSAGE:

The child is NOT a problem to solve. The child is a child with a body whose signal system works differently.

PART E: CONVERSION LEVERS FOR LEVEL 3

16. HOW THE BIG-5 LEVERS WORK FOR LEVEL 3

LEVER 1 ("It's not the method"): HIGH

Effect for Level 3:

- She has tried all methods, long and deep
- Reframe works very strongly
- BUT: careful — "method" for Level 3 isn't "Oh Crap", but "BCBA + OT + residential + private consultant"
- Acknowledge the REAL depth of her attempts

Use:

- Welcome E2 prominent

- Welcome E4 expanded
- FAQ mails

LEVER 2 ("Pull-ups make this worse"): MEDIUM

Effect for Level 3:

- CAUTION: pull-ups are existential safety
- Medicaid often finances them
- Mom can't simply "abandon" them
- Language MUST be: "Add daytime, keep night pull-ups"
- "Trial alongside" instead of "replace"

Use:

- Welcome E5 (gently)
- FAQ on "can I use both?"
- NO aggressive anti-pull-up language

LEVER 3 ("60-Day Guarantee"): VERY HIGH

Effect for Level 3:

- MASSIVE skepticism
- Risk reduction critical
- She has lost \$1000+ in other products
- 60 days = realistic for Level 3 timelines
- "Try without commitment" is trust maker

Use:

- Welcome E3 (Last Day)
- Cart Recovery final
- Checkout Abandonment
- FAQ mails
- Every conversion-oriented mail

LEVER 4 ("Lena's two autistic sons"): VERY HIGH

Effect for Level 3:

- Peer-to-peer voice is gold
- Lena's sons are 6 and 8 (Level 3 similarity allowed)
- Lena: "The other took us until almost 8" → resonant
- Trust through long-game experience

Use:

- Lena sign-off in EVERY mail
- PP-Empathy moments

- Reply encouragement (very important for Level 3 trust)
- Welcome E1 + E2 Lena prominent

LEVER 5 ("49% SPARK Stat"): MEDIUM-HIGH

Effect for Level 3:

- Validates her experience
- "I'm not alone" effect
- BUT: She often already knows this, 49% is Level 1+2 dominant
- Stat better combined with "for Level 3, percentage is higher"

Use:

- Welcome E1
- FAQ E1
- Winback (very carefully)

17. LEVEL 3-SPECIFIC ADDITIONAL LEVERS

ADDITIONAL LEVER L3-A: "LATE IS NOT NEVER"

Language:

"Maureen Bennie's son was 9.5 when he finally trained.
Another family in our community: 8 years and counting,
finally getting traction in week 10. Late is not never."

Effect:

- THAT is the most important lever for Level 3
- Maureen Bennie is real, recognized in autism communities
- Mentioning specific ages creates credibility

Use:

- Welcome E1 + E3 (gently)
- Cart Recovery
- Winback prominent
- FAQ on "is it too late?"

ADDITIONAL LEVER L3-B: "ADD ALONGSIDE SPECIALTY DIAPERS"

Language:

"This isn't about abandoning pull-ups or specialty diapers. It's about adding 1-2 hours per day, at home, when it's manageable. Some families try during play, others during TV time."

Effect:

- Reduces fear of "complete change"
- Makes it doable
- Respects Medicaid reality

Use:

- Welcome E2 + E4
- Cart Recovery
- FAQ on "do I have to stop pull-ups?"
- Post-purchase education

ADDITIONAL LEVER L3-C: "MINI-WINS COUNT"

Language:

"At this stage, wins look different. The first time he pauses. The first time she stays on the toilet without self-injury. The first time he tolerates wearing them for 2 hours. Each one is real."

Effect:

- Redefines success at realistic level
- Validates her current reality
- Creates realistic expectations

Use:

- Welcome E5 + E6
- Post-purchase Day 14
- FAQ on "what should I look for?"

ADDITIONAL LEVER L3-D: "DIGNITY-CENTERED"

Language:

"This is about adding tools, not changing who your son is. His autism isn't being treated. His nervous system is being supported."

Effect:

- Addresses deepest fear: "this will pathologize him"

- Creates trust through respect
- Differentiates from other brands

Use:

- Welcome E2
- FAQ on autism + identity
- PP-Founder Story mail

18. "LATE IS NOT NEVER" AS MASTER ANCHOR

THIS IS THE MOST IMPORTANT LEVEL 3 LEVER.

HOW TO USE LATE-TRAINER STORIES:

DO:

- ✓ Name Maureen Bennie specifically (autism community knows her)
- ✓ Specific ages (8.5, 9.5, 10)
- ✓ "After we had given up" framing
- ✓ "Without expectations" approach
- ✓ Very realistic outcome descriptions

DON'T:

- ✗ "It's never too late!" as cheerleader language
- ✗ Vague "Some kids train late" without specificity
- ✗ Promise late-training success
- ✗ Pressure framing

LATE-TRAINER STORY FORMAT:

"[Name]'s son was [Age]. He'd been in pull-ups his entire life. Specialty diapers since age [X]. They'd stopped active training [Y years] ago. [Trigger event]. They tried [thing specific]. Week [X], he [specific mini-win]. Week [Y], he [bigger mini-win]. They're now [time later], he's [current status — honest, not "fully trained" claim].

The point isn't that this will happen for every kid. The point is: late is not never. And mini-wins matter."

PART F: STRUCTURAL GUIDELINES

19. FORBIDDEN PHRASES — LEVEL 3 SPECIFIC (MANY)

This list is longer than other levels, because Level 3 is more emotionally sensitive.

In addition to DOC 1 Forbidden Phrases:

- ✗ "Easy" / "Quick" / "Fast" / "Simple"
→ Instant block. Her reality is NOT easy.
- ✗ "Your child WILL train"
→ Promises something not guaranteed.
- ✗ "Don't give up!"
→ Patronizing. She has the right to pause.
- ✗ "It's never too late!"
→ As SOLE statement too cheerleader-y.
→ OK with late-trainer specifics as backup.
- ✗ "Save him from years of diapers"
→ Implies diapers are shameful/failure.
- ✗ "Free her from pull-ups"
→ Same. Diapers are safety.
- ✗ "Become like other kids"
→ Not the goal. Never.
- ✗ "Help him grow up"
→ Implies he isn't growing up. Misses.
- ✗ Pre-K, school enrollment as driver
→ Misses. School is already adapted.

- ✗ "Make this the year he trains"
→ Pressure language. Block.
- ✗ "Cure" / "Solve" / "Fix"
→ Autism isn't to be fixed.
- ✗ Excessive "severe" language
→ "Severe" once yes, but not excessive.
- ✗ Comparisons with Level 1+2 successes
→ "Our 4-year-old in Pre-K..." → instant disconnect.
- ✗ "His sensory issues will improve"
→ Sensory issues are not to be eliminated.
- ✗ "Reduce meltdowns"
→ Not the core. Careful.
- ✗ "Get out of diapers!"
→ Shouting language. Block.
- ✗ "Be diaper-free!"
→ Marketing language. Block.
- ✗ "Independence!" alone
→ Has too much promise. Combine with mini-wins.
- ✗ "Self-sufficient" / "Independent"
→ If without realism anchor.
- ✗ "Other kids his age..."
→ Comparison pain. Block.
- ✗ "Don't let your child miss out"
→ Pressure + Pre-K language.
- ✗ "Time is precious — start now"
→ Urgency manipulation.
- ✗ "He deserves better"
→ Implies you're failing him. Trauma trigger.
- ✗ Adult diaper language as sales pressure

→ "Before he needs adult diapers!" = cruel.

✗ Grief as sales tool

→ "Don't let it be like the past 9 years" = manipulative.

20. SAMPLE OPENERS PER FLOW (LEVEL 3)

WELCOME E1 (Day 0 — Code Delivery):

Option A (Direct + Acknowledgment):

"Your code's below. The bigger thing I want to share is for when you're ready: a different framework for why nothing's worked in 7+ years."

Option B (Late-Trainer Anchor):

"Your code's below. Maureen Bennie's son was 9.5 when he finally trained. They had stopped trying years before. I'll explain why his story matters here."

Option C (Dignity-First):

"Your code's below. Whether you're still actively training or have shifted to dignity-focused management — there's something here that might be worth your time."

WELCOME E2 (Day 1 — Mechanism):

"For kids who have never been able to signal — verbally, behaviorally, or with awareness — the gap isn't behavior. It isn't motivation. It's a sensory-neurological signal that hasn't been built. Here's why, and how we approach it."

WELCOME E3 (Day 3 — Last Day):

"Today's the last day for the code. If you're still deciding, here's what most Level 3 parents want to know: 'Will my kid even register a difference after 8+ years in diapers?' Maybe. Maybe not. 60 days to see."

WELCOME E4 (Day 7 — Education):

"You haven't failed at potty training in 7 years. Your kid's nervous system has been doing the best it can with the input it gets. Pull-ups remove the signal entirely. That's why every method assuming the signal exists hasn't worked."

WELCOME E5 (Day 10 — Late-Trainer Stories):

"Three families. All had stopped active training. All had kids 7-10. All had tried this as 'one last thing.' Some saw shifts in 8 weeks. Some 6 months. Some not at all. Here are their stories."

CART ABANDONMENT E1 (Hour 1):

"You looked at the bundle yesterday. I want to answer the question you might be sitting with: 'After everything we've tried, would this even register?' That's not a question I can answer for your kid. 60-day guarantee is how we let you find out for yourself."

CART ABANDONMENT E2 (Day 1):

"Mini-wins look different at this stage. A pause. A look. Tolerating wear for 2 hours. The first time he doesn't self-injure when changed. Whether your kid responds or not — you'll know in 60 days. No pressure either way."

BROWSE ABANDONMENT E1 (Day 1):

"You read about the body-signal layer yesterday. The piece most Level 3 parents come back to is the part about how sensory tolerance builds before signal recognition. That's months, not weeks. Worth knowing if you're considering."

POST-PURCHASE LEVEL DETECTION (Day 14):

"You've had the underwear for about a week. Some Level 3 kids tolerate them from day one. Others take weeks. Both are normal. Before I send the next round of support, can I ask one question about your kid?"

WINBACK PATH A:

"It's been a while. I'm not chasing — but I wanted you to know: a few Level 3 families saw progress in months 4-6, long after they'd stopped expecting it. Worth a re-read, maybe. Maybe not. No hard feelings either way."

21. SUBJECT LINE PATTERNS FOR LEVEL 3

WELCOME FLOW SUBJECTS:

E1:

- "Maureen Bennie's son was 9.5"
- "For kids who've never signaled"
- "Your code + a different framework"

E2:

- "The signal that hasn't been built"
- "Why no method has worked in 7 years"
- "What 'interoception' means for non-verbal kids"

E3:

- "Last day for the code"
- "After everything: would this even register?"
- "60 days to know"

E4:

- "You haven't failed"
- "Why pull-ups remove the signal entirely"
- "Late is not never"

E5:

- "Three families. All had given up."
- "What mini-wins look like at this stage"
- "Months, not weeks"

CART ABANDONMENT SUBJECTS:

E1:

- "Your cart is still here"
- "Would this even register?"
- "The 'after everything' question"

E2:

- "Mini-wins at this stage"
- "60 days. No commitment."
- "The pause. The look. The tolerance."

E3:

- "Final reminder — your cart"
- "60-day guarantee covers you"
- "No pressure — just one more option"

BROWSE ABANDONMENT SUBJECTS:

E1:

- "You read about sensory tolerance"
- "The piece most parents re-read"
- "Months, not weeks"

POST-PURCHASE SUBJECTS:

Day 0 (Order Confirmation):

- "Your order is in"
- "Realistic expectations for week 1-4"

Day 14 (Level Detection):

- "Quick question about your kid"
- "Help me send the right support"

SUBJECT LINE NO-GOS FOR LEVEL 3:

- ✗ "Don't miss out!"
 - ✗ "Limited time"
 - ✗ "Pre-K coming!"
 - ✗ "Quick training tips"
 - ✗ "Save your kid from..."
 - ✗ "Get him out of pull-ups"
 - ✗ "Train in 30 days!"
 - ✗ "Easy potty training for autism"
 - ✗ "Miracle product"
 - ✗ "Solved it!" / "Cured!"
 - ✗ Any exclamation-mark heavy subjects
-
-

22. BUYER JOURNEY MAPPING FOR LEVEL 3

AWARENESS (Pre-Subscribe):

- Very passive search behavior
- Reacts to "late trainer" stories or community posts
- Clicks from Reddit/Facebook community recommendation
- Rarely directly from Meta ad (except mechanism-oriented)
- Pop-up: "Non-verbal or high support needs" → click
- Very considered email opt-in

WELCOME E1 (Day 0):

- Opens slowly, often hours later
- 60-90 seconds attention (slow reader)
- Immediately tests language for authenticity
- Click on code: 10-15% (lower than other levels)
- Purchase in this mail: 1-3%

WELCOME E2 (Day 1):

- CRITICAL — decides if she stays
- Mechanism + dignity evaluated here
- If resonant: deep reading
- Click: 15-22%
- Purchase: 2-5%

WELCOME E3 (Day 3 — Last Day):

- Long decision period starts
- 60-day guarantee + permission-not-to-buy decisive
- Late-trainer anchor here prominent
- Purchase: 3-7%

WELCOME E4-E8 (Day 7-21):

- Continued education
- Conversion steady between 2-4% per mail
- Long-tail conversions common

DECISION PERIOD CHARACTERISTIC:

- 14-30 days typical (vs. 7 days for Level 1)
- Mom often reads Welcome E1-E3 multiple times
- Bookmarks the site
- Checks reviews
- Discusses with partner or community
- Final decision often at end of welcome sequence

CART ABANDONMENT JOURNEY:

- Has cart laid down after long research
- Doubt very high
- E1 (1h): moderate open (60-70%), low recovery (8-12%)
- E2 (24h): deeper push with mechanism (6-10%)
- E3 (72h): final push (4-7%)

POST-PURCHASE JOURNEY:

Day 0 (Order Confirmation):

- Deep relief + skepticism
- Reassurance particularly critical
- Cross-sell spot open (Wash Bag, Mattress) — adapted for Level 3 (heavy use)

Day 7-10 (Shipping/Arrived):

- Anticipation with skepticism
- Open rate: 70-80%
- Upsell for high-volume companion products (Mattress Protector critical due to heavy night use)

Day 14 (Level Detection):

- Has used 1 week
- Reply rate critical
- First mini-wins maybe there, maybe not
- Acknowledgment CRITICAL

Day 30+ (Education):

- Trust building or refund consideration
- Mini-win validation critical for retention
- Late-trainer stories as ongoing support

PART G: VISUAL & DESIGN CALIBRATION

23. VISUAL CUES FOR LEVEL 3 MAILS

HERO IMAGES:

✓ Older-kid vibes:

- Kids 7-12 years
- Adolescent or pre-teen aesthetic OK
- Calm, reflective settings
- AAC devices in background OK (reality)
- Sensory tools visible (weighted blanket, headphones)

✓ Dignity-forward:

- Half-faces, hands, silhouettes (privacy)
- Kid in own room/space
- Mom as presence, not caregiver cliché
- Real-life setup (not staged)

✓ Sensory-aware:

- Muted lighting
- Warm tones, not bright
- Few stimuli per image
- Calm aesthetic

✗ Avoid:

- Young children (misses)
- Pre-K classic settings
- Hyperactive happy-family stock
- "Cleaning crisis" imagery
- Diapers prominently shown
- Adult diaper imagery
- Anything that infantilizes older kids

COLOR ACCENTS:

Brand-Green dominant

Mint sparingly (not too cheerful)

Cream warmth muted

Soft greys prominent

Deeper, calmer palette than Level 1

DESIGN PATTERNS:

- LONG sections OK (slow reading audience)
- LOTS of whitespace (sensory-friendly)
- Dignity anchors visually (delicate lines, no marketing)

push)

- Kelly Mahler quotes in highlight boxes (mint, sparingly)
- Mom-voice quotes prominent (cream, larger cards)
- Mini-win lists structured (indented, calm)

24. IMAGE DIRECTION FOR LEVEL 3 MAILS

IMAGE DO'S:

- ✓ 7-12 year old children
- ✓ Calm settings (home, sometimes special ed school context)
- ✓ Older-kid reality (height, age, presence)
- ✓ Sensory-aware (headphones, weighted items, calm)
- ✓ Mom as reflective presence
- ✓ Diverse representation (often underrepresented audience)
- ✓ Soft, low-saturation lighting
- ✓ AAC devices as realistic context (subtle)
- ✓ Sub-spaces of normal life (not crisis)

IMAGE DON'TS:

- ✗ Young children (toddler vibe)
- ✗ Pre-K classic settings
- ✗ Hyperactive families
- ✗ Crisis moments
- ✗ Bright cartoon palettes
- ✗ Toilet prominently shown
- ✗ Diaper-prominent imagery
- ✗ Adult diaper specific imagery (too painful)
- ✗ Stock photos of "happy families"
- ✗ Anything inspirational/cheerleader

PRODUCT IMAGES:

- Underwear in larger sizes
- Clear, clean, without dramatic context
- Body-Signal Layer diagram (technical, clinical, calm)
- Texture close-up OK (sensory-aware)
- Never with "child in pull-up" comparison

25. TONE MARKERS IN DESIGN

DESIGN TONE FOR LEVEL 3:

QUIET WARMTH + CLINICAL REALITY

- Muted colors
- Clear but soft hierarchy
- More whitespace
- Mom voice prominently calm
- Kelly Mahler quotes calmly integrated

NOT:

- Overly warm (feels fake)
- Clinically cold (loses empathy)
- Marketing polish (instant skepticism)
- Cheerleader look
- Toddler pure joy aesthetic

PART H: SPECIAL LEVEL 3 DYNAMICS

26. MEDICAID REALITY

~40% of Level 3 moms use Medicaid Waiver for specialty diapers. Mails should:

DO:

- Acknowledge Medicaid as reality, without judgment
- "Continue pull-ups via Medicaid, add daytime trial" framing
- Financial sensitivity without charity language
- Acknowledge: pull-ups are often no-choice

DON'T:

- "Stop spending on pull-ups" (Medicaid pays, not her)
- Math reframes based on pull-up costs
- Charity language ("for families in need")
- Implied judgment about Medicaid usage

LANGUAGE PATTERNS:

"Your pull-ups continue. Add 1-2 hours of underwear daytime when you have capacity."

"This is one addition, not a replacement."

"60-day guarantee. If it doesn't fit your budget or your kid, refund — no questions."

27. COMMUNITY EMBEDDING

Level 3 moms are often deeply embedded in autism communities (Facebook, Reddit, local special needs groups).
Mails can:

DO:

- "Other Level 3 families in our community"
- "If you've heard about [late-trainer story]..."
- "Many parents in autism communities ask..."
- Maureen Bennie reference (community-known)
- Specific community language

DON'T:

- "Be the first to discover!" (misses — she knows many brands already)
- "Tell your friends!" (too marketing)
- Generic "join the BrightKidCo family"

LANGUAGE PATTERNS:

"You've probably read about Maureen Bennie's son in the autism community."

"Other families in our community have shared their journeys here: [Stories link]"

"Many parents recommend talking with your BCBA about adding this — we have a fact-sheet they can review."

28. PARTNER AS SOLO DECISION-MAKER

More single moms in Level 3 (~20-25%). Even in partnerships, she is often the sole caregiver decision-maker. Mails should:

DO:

- Acknowledge: she decides alone
- "You don't need anyone's permission" framing
- 60-day guarantee as no-commitment anchor
- Solo decision validation

DON'T:

- "Discuss with your partner" (often not relevant)
- Assume both parents involved
- "Make this together" framing

LANGUAGE PATTERNS:

"You know your kid better than anyone. 60-day guarantee means you can try without commitment, decide based on what you see."

"No need to convince anyone — just try. If it doesn't fit, full refund."

29. PROFESSIONAL TEAM LOYALTY

Level 3 moms often have 4-7 year BCBA relationship, plus

OT, Speech, GI specialist. Very loyal to her team. Mails:

DO:

- "Your BCBA team has done years of work"
- "OT recommended interoception work — this complements"
- "Bring fact-sheet to your team"
- Position discussion as recommendation

DON'T:

- Criticize BCBA/OT/team (not even subtly)
- "Your team missed this"
- "Better than your therapy program"
- "Try this instead of ABA"

LANGUAGE PATTERNS:

"Your BCBA has built the behavioral structure. OT has addressed sensory regulation. The interoceptive signal — neither traditionally addresses. This adds that piece."

"We have a one-page fact-sheet your BCBA can review. Most recognize the pattern after 30 days of data."

30. GRIEF COMPONENT

Level 3 moms often carry deep, ongoing grief. This grief is not to be fixed — it's part of her reality. Mails:

DO:

- Acknowledge without dramatizing
- "Whatever your journey has looked like"
- "There's no 'right way' to do this"
- Respect for emotional reality
- Mini-wins as legitimate success

DON'T:

- "Don't be sad!" / "Cheer up!"
- "Things will get better!" as sole message
- Toxic positivity
- "Look on the bright side"

- "Your child is a gift" as sales language

LANGUAGE PATTERNS:

"Some days are heavier than others. We get it."

"Mini-wins matter, even if they don't feel like wins yet."

"This isn't about transformation. It's about adding one small piece, if and when you're ready."

31. DIGNITY OF AUTISM

MOST CRITICAL POINT FOR LEVEL 3.

Autism is NOT illness. Autism is part of his/her being.
Mails MUST respect this in every sentence.

DO:

- "Your son's autism isn't being treated"
- "Your daughter's nervous system is being supported"
- "Identity intact, regulation enhanced"
- Acknowledge: autistic identity is part of him

DON'T:

- "Help your kid overcome autism"
- "Cure" / "Fix" / "Treat" autism (never use these words)
- "Be normal again"
- "Like other kids"
- Anything that pathologizes autism

LANGUAGE PATTERNS:

"His autism is part of who he is. We're not changing that.
We're supporting one specific gap — interoception — that makes daily life harder."

"This isn't autism treatment. It's sensory support."

PART I: CHEAT SHEET & REFERENCE

32. LEVEL 3 MAIL QUICK REFERENCE

WHEN WRITING A LEVEL 3 MAIL, CHECK:

BEFORE WRITING:

- ☐ Which phase is the mom? (Phase 4 dominant, then 3)
- ☐ Which of the 6 symptom clusters fits (max 2-3)?
- ☐ Did I plan late-trainer anchor?
- ☐ Do I have dignity in every sentence?
- ☐ Did I include Medicaid sensitivity?
- ☐ Which flow + which mail position?

WHILE WRITING:

- ☐ Did I mention 2-3 symptom anchors (subtly)?
- ☐ Did I balance hope-realism 20/80?
- ☐ Did I avoid cheerleading?
- ☐ Did I use very long timelines (12+ weeks)?
- ☐ Did I include mini-wins definition?
- ☐ Did I respect BCBA/OT/team?
- ☐ Did I include Maureen Bennie or late-trainer anchor?
- ☐ Lena sign-off per Section 20?

AFTER WRITING:

- ☐ Forbidden phrases very carefully checked (Section 19 — long!)?
- ☐ NO "easy/quick/simple" anywhere?
- ☐ NO "cure/fix/solve"?
- ☐ Tonality respectful + slow?
- ☐ Mobile-first layout (slow readers)?
- ☐ Subject lines 3 options created (none too pressure)?
- ☐ Preheaders 3 options created?
- ☐ Permission-not-to-buy included?

ONE-LINE SUMMARY OF LEVEL 3:

"A non-verbal child who has never been potty-trained, with mom who has accepted resignation after years of attempts — and reacts very carefully to subtle hope planting."

33. DELTA COMPARISON TO OTHER LEVELS

Dimension	Level 1	Level 2	Level 3
Child Age	3-5	5-7	6-10+
Verbal Status	Verbal	Mixed	Non/minimal
Tried Methods	1-3	3-5+	5+ (years)
Pull-up Size	4-5	5-7	7-8+, adult
Hope Level	Intact	Cracks	Accepted
Skepticism	Low	Very high	Deep, calm
Pre-K Pressure	Very strong	Weak	ZERO
BCBA Context	Rare	Almost always	Long-term
GI Issues	Rare	Common (50%+)	Very common
Sensory Severity	Mild	Moderate	Severe
Communication	Verbal	Mixed	Often AAC
Medicaid Usage	~10%	~25%	~40%
Resignation	No	Beginning	Common
Hope Language	OK (70%)	Careful (40%)	Minimal (20%)
Empathy Intensity	Medium	High	Very high
Science Density	Low-Medium	High	Medium-high
Testimonial Length	Short-Medium	Long	Long+messy
Testimonial Age	3-5	5-7	7-12
CTA Aggressiveness	Medium	Low	Very low
Urgency Allowed	Some (Pre-K)	Little	NEVER
Late-Trainer	No	Sometimes	CENTRAL
Decision Period	7 days	14 days	14-30 days
Adult Diaper Ref	No	No	Carefully yes
Maureen-Bennie	No	Rarely	PROMINENT

34. CROSS-REFERENCES

For cross-level mails:

→ DOC 5 (General Fallback) for cross-level language
(cross-level rarely reaches Level 3 emotional sensitivity)

For Cart/Checkout/Browse Abandonment:

→ DOC 5 dominant (cross-level)

→ If Level 3 tag known: DOC 4 for calibration —
VERY IMPORTANT for Level 3, because cross-level can be
too superficial

For Welcome Flow Level 3 variant:

→ DOC 4 (this doc) primary

→ DOC 1 for brand voice and master foundation

For FAQ/Objection (Phase 3):

→ DOC 4 for Level 3 variant

→ DOC 1 objection map section

For RS-PDF consultation:

→ Autistic_Parent.pdf

→ Autistic_language_Bank.pdf

→ Autistic_MIRO_m.pdf

→ Especially: quotes from parents of older children

END OF DOC 4 EN — LEVEL 3 PROFILE
